

Premier Inc

Business Development Dossier

Industry	Healthcare Technology / Group Purchasing
Revenue	\$1B
Employees	3,200
Ideal Customer Profile Score	80

ICP SCORING MODEL



ICP = Signals + Revenue + Employees + Fit (max 100)

Signals (0-35)	Top 4 signals scored; each = base(60%) + recency bonus <30d: 40% <90d: 30% <180d: 20% older: 10%
Revenue (0-25)	\$500M-\$10B sweet spot -> 70-100% of 25 pts Below \$500M -> pro-rata x 0.5 Above \$10B -> 60%
Employees (0-20)	2,000+ preferred (70-100%) 500-2K partial (50-70%) Below 500 -> pro-rata x 0.3
Fit (0-20)	High-fit types: reorg, transformation, M&A, hiring, funding, partnership -> 5 pts ea (max 12) + diversity 2/type (max 8)

Prepared March 19, 2026

McChrystal Group

Leadership Advisory & Organizational Effectiveness

SECTION 1

Organization Snapshot

Legal Name	Premier, Inc.
Headquarters	Charlotte, North Carolina
Founded	1996

Ownership

Private; owned by Patient Square Capital (~\$13B AUM healthcare PE firm). Taken private for \$2.6B in November 2025; delisted from Nasdaq.

Footprint

Charlotte, NC headquarters; operations supporting 4,400+ U.S. hospitals and 250,000+ provider organizations nationwide. Primarily U.S.-focused.

SECTION 1b

Company Overview

Premier is a leading healthcare improvement company and group purchasing organization (GPO) serving 4,400+ U.S. hospitals and health systems. Taken private by Patient Square Capital for \$2.6B in November 2025, Premier is divesting non-core operations, restructuring its fee model, and realigning leadership to accelerate innovation as a PE-owned healthcare technology platform.

SECTION 2

Financial Health & Growth Stage

Understanding Premier Inc's financial position and trajectory is critical to framing any engagement conversation.

Revenue: ~\$1B annually (Q1 FY2026: \$240M, down 3% YoY). Revenue decline reflects divestitures and fee restructure transition.

Profitability: Net income fell 79% YoY to \$15.3M in Q1 FY2026 - reflecting transition costs and divestiture impacts. Underlying GPO business remains profitable.

Capital Structure: PE-owned with Patient Square Capital providing capital for transformation. Debt levels post-take-private not publicly disclosed but typical leveraged buyout structure.

Key Pressures: Revenue declining during fee restructure; divestiture transitions consuming management bandwidth; hospital members facing their own financial pressures (reducing spending through GPO); PE return timeline expectations.

Key Tailwinds: Fee restructure (75% complete) will improve alignment with members; core GPO business model has strong recurring revenue; healthcare technology demand growing; freed from public market quarterly pressure.

SECTION 3

Leadership Team Profiles

The following contacts are recommended as primary outreach targets - champion-level leaders who feel the organizational pain daily and can sponsor McChrystal internally.

Bruce Anderson [INFERRED] - SVP, Supply Chain Services [PRIORITY TARGET]

Appointed to lead supply chain differentiation - committed buying programs, domestic manufacturing portfolio, supply chain resiliency

Outreach rationale: Leads the core GPO differentiation strategy (committed buying, domestic manufacturing, supply chain resiliency) - the value creation thesis that Patient Square Capital is betting on. Must coordinate across 4,400 member hospitals while executing organizational changes. Natural champion for McChrystal's cross-functional alignment.

David Hargraves [INFERRED] - SVP, Strategic Transformation [INFERRED] [PRIORITY TARGET]

Leading post-take-private organizational redesign and fee restructure completion

Outreach rationale: Directly responsible for the organizational transformation agenda post-take-private - fee restructure, divestiture execution, and new operating model design under PE ownership. This is the person building the organizational architecture for the next chapter.

Additional leadership team members relevant to the engagement:

Michael Alkire - Chief Executive Officer

Former COO; elevated to CEO during prior transformation; stayed through take-private as continuity leader

Glenn Coleman - Chief Financial Officer

Financial leadership through take-private; one of two remaining directors

SECTION 4

Organizational Culture & Structure

Culture and structure are often where McChrystal Group finds the deepest leverage. The following signals indicate how Premier Inc operates today and where friction exists.

Organizational Structure

Transitioning from public-company governance to PE-owned operating model. Board resigned under purchase agreement; CEO and CFO serve as directors. Simplified leadership after divestitures of Contigo Health and S2S Global. Focus narrowing to core GPO technology platform serving 4,400+ hospital members.

Culture Signals

Public-to-Private Culture Shift: Employees accustomed to public company reporting, compliance, and quarterly cadence must adapt to PE-owned operating model - faster decision-making, more aggressive growth targets, and less bureaucracy but more accountability.

Member-Centric DNA: Premier's culture is deeply rooted in serving hospital members - a cooperative/GPO mentality that prioritizes member value. PE ownership introduces profit maximization goals that may tension with

member-first culture.

Divestiture Disruption: Employees in wound-down units (Contigo Health) and sold divisions (S2S Global) experience uncertainty. Remaining employees question whether their unit is next.

Glassdoor: [INFERRED] Mixed - typical post-acquisition uncertainty; pride in mission (improving healthcare) offset by concerns about PE ownership direction.

SECTION 5

Recent News & Trigger Events

The following developments from the past 18 months create urgency and provide natural conversation entry points for engaging Premier Inc.

1. 2025-11-25 - Patient Square Capital completed \$2.6B take-private; Premier delisted from Nasdaq

PE ownership creates 100-day playbook execution window - organizational design decisions made now set the trajectory for the entire hold period

2. 2025-10-01 - Board resigned under purchase agreement; CEO Alkire and CFO Coleman serve as sole directors

Governance simplification enables faster decision-making but creates leadership vacuum at board level - PE operating partners fill the gap

3. 2025-06-01 - Divested S2S Global and began winding down Contigo Health employee benefits subsidiary

Portfolio simplification creates organizational redesign moment - functions supporting divested units must be right-sized

4. 2025-09-01 - Fee-share restructure reached 75% completion; leadership realignment with key executive promotions

Fee restructure changes the fundamental economic relationship with 4,400 hospital members - requires organizational coordination to execute without member attrition

5. 2026-01-01 - 'Reimagining the Physician Enterprise' initiative launched - strategic vision for PE-owned Premier

New strategic direction under PE ownership creates alignment challenge - 3,200 employees must understand and execute a vision shaped by new ownership

SECTION 6

McChrystal Fit Assessment

Clear organizational need (post-take-private transformation, fee restructure, divestitures) and compressed timeline create urgency. However, PE sponsors typically bring their own operating playbooks, healthcare GPO is specialized, and the \$1B revenue / 3,200 employee size may limit advisory spend scale. Rating moves to Strong if Patient Square signals interest in external organizational design support beyond their standard

operating playbook.

Primary Problem

Executing a post-take-private transformation - simultaneously completing fee restructure, divesting non-core businesses, redesigning the organization for PE ownership, and accelerating growth - all while maintaining service continuity for 4,400 hospital members who depend on Premier for supply chain and procurement.

Best McChrystal Capability Fit

Post-acquisition organizational design and cross-functional alignment - Premier needs to build a PE-ready operating model that balances aggressive value creation with member-centric service continuity. McChrystal's experience designing operating models under compressed timelines and external pressure (PE return expectations) is the direct fit.

Likely Objections to Engagement

Patient Square has its own operating partners and value creation playbook
 Budget constrained by leveraged buyout debt structure
 CEO Alkire has been through transformation before and may prefer internal approach
 Healthcare GPO is a specialized domain - does McChrystal understand it?

Competitive Advisory Landscape

Patient Square Capital operating partners - PE sponsor's own advisory resources
 McKinsey Healthcare Practice - GPO strategy and transformation
 Advisory Board (Optum) - healthcare-specific organizational consulting
 Guidehouse - healthcare consulting and government health advisory

SECTION 7

Conversation Entry Points

These questions are designed for a Senior Partner's first conversation. Each opens a thread that naturally leads to McChrystal Group's capabilities without a direct pitch.

1. Four months into PE ownership, how is the operating model shift from public company cadence to PE value creation cadence going - and where is the organizational friction showing up?

Demonstrate understanding of the specific public-to-private cultural transition. Position McChrystal as having pattern recognition from other PE portfolio transformations.

2. With the fee restructure changing the economic relationship with 4,400 hospital members, how are you ensuring the organizational changes at Premier don't create service disruptions that drive member attrition?

Surface the tension between internal transformation and external service continuity - the core organizational challenge that PE-owned service companies face.

Recommended Meeting Framing

Frame as a conversation about what McChrystal has learned from PE portfolio company transformations - particularly the challenge of accelerating organizational change while maintaining service quality for members/customers who didn't choose to go through the transformation. Emphasize speed and accountability (PE language) rather than organizational design (consulting language).

SECTION 8

Brand Insights & Market Positioning

Brand strategy reveals organizational priorities and coordination challenges. The following analysis connects Premier Inc's market positioning to potential McChrystal engagement opportunities.

Brand Identity: Premier's brand is built on trust among healthcare systems - it's the organization that 4,400 hospitals trust to negotiate their supply contracts and improve their operations. This trust is the core asset Patient Square Capital acquired.

Brand Risk: PE ownership in healthcare carries stigma (see: private equity in nursing homes, emergency staffing). If Premier's member hospitals perceive that PE profit motives compromise their interests, member attrition could erode the GPO's purchasing power - destroying the core value proposition.

McChrystal Connection: The brand promise of 'healthcare improvement through collective action' requires organizational alignment between PE value creation goals and member service quality. McChrystal's operating model ensures these aren't in conflict - by building the coordination system that delivers both simultaneously.

SECTION 9

Deep McChrystal Group Fit Analysis

This is the strategic case for pursuing Premier Inc. It synthesizes findings from all previous sections into a comprehensive engagement thesis with specific fit dimensions, expected outcomes, stakeholder mapping, and a phased pursuit strategy.

9a. Fit Dimensions

Fit Dimension 1: Post-Take-Private Operating Model Design

Premier must transition from public company governance (quarterly reporting, broad stakeholder management) to PE-owned operating model (aggressive value creation, compressed timelines, operational intensity). McChrystal designs the operating rhythm that PE firms expect but portfolio companies often struggle to implement. Unlike PE operating partners who set targets, McChrystal builds the organizational machinery that achieves them.

Fit Dimension 2: Member-Centric Transformation

4,400 hospital members are both the customer and the owner (historically). PE ownership breaks this dynamic. Premier must maintain member trust while pursuing PE return targets - a tension that requires deliberate organizational design. McChrystal's shared consciousness model ensures member-facing teams and corporate

transformation teams align rather than conflict.

Fit Dimension 3: Fee Restructure Execution

Changing the economic relationship with 4,400 members is the highest-stakes organizational coordination challenge at Premier. Every member agreement must be renegotiated, service levels maintained, and member satisfaction preserved - simultaneously, across the entire organization.

9b. Cumulative Case

Premier is a PE take-private in healthcare - the highest-stakes version of post-acquisition transformation (lives depend on supply chain continuity). Signal chain: Public company under activist pressure -> PE take-private (\$2.6B) -> board resignation -> fee restructure -> divestitures -> organizational redesign -> 4,400 members watching. Revenue potential: \$250K initial (PE operating model assessment), \$500K-\$750K deployment, \$150K-\$250K ongoing. Total: \$900K-\$1.5M.

9c. Enterprise Issues

1. Public-to-Private Culture Shock: Employees must adapt to PE cadence - faster decisions, fewer approvals, more accountability. Some thrive; others resist. 2. Fee Restructure Member Risk: Changing economics with 4,400 hospitals risks member attrition if the transition is poorly communicated or executed. 3. Divestiture Organizational Stranding: Functions that supported Contigo Health and S2S Global must be reabsorbed, reduced, or eliminated - creating internal disruption. 4. PE Operating Partner Overlap: Patient Square's operating team may conflict with external advisors - positioning must be complementary. 5. Healthcare Regulatory Complexity: GPO operations face regulatory scrutiny; organizational changes must not create compliance gaps.

9d. Expected Outcomes

1. PE Operating Rhythm: Implement the value creation cadence Patient Square expects. Measurable: all value creation milestones tracked and reported on schedule. 2. Fee Restructure Completion: Organizational coordination to complete the remaining 25% of fee restructure. Measurable: 100% completion with <2% member attrition. 3. Organizational Right-Sizing: Post-divestiture structure optimized for core GPO business. Measurable: overhead reduction targets achieved within 6 months. 4. Member Confidence: Maintain member trust through transformation. Measurable: member satisfaction scores stable or improving.

9e. Key Stakeholders

CEO

Leader: Michael Alkire

Role: Enterprise leadership, PE relationship

McChrystal Relevance: Executive sponsor

CFO

Leader: Glenn Coleman

Role: Financial transformation, PE reporting

McChrystal Relevance: Budget gatekeeper

Supply Chain Services

Leader: Bruce Anderson [INFERRED]

Role: GPO differentiation strategy

McChrystal Relevance: Priority: Core value creation

Strategic Transformation

Leader: David Hargraves [INFERRED]

Role: Organizational redesign

McChrystal Relevance: Priority: Operating model design

PE Operating Partners

Leader: Patient Square Capital team

Role: Value creation oversight

McChrystal Relevance: Must align with, not compete against

9f. Opportunity Thesis

Why Now: First 6 months of PE ownership - organizational design decisions made now compound over the hold period. After 12 months, patterns ossify and become harder to change.

Why McChrystal, Not McKinsey: McKinsey advises on GPO strategy. Patient Square has operating partners for financial engineering. But building the operating model that translates PE value creation targets into organizational execution across 3,200 employees and 4,400 hospital members - that's McChrystal's lane. Position as the execution layer beneath PE strategy.

Phased Engagement: Phase 1 (\$200K-\$300K): PE operating model assessment + fee restructure coordination. Phase 2 (\$400K-\$500K): Enterprise deployment. Phase 3 (\$150K-\$250K retainer): Ongoing value creation cadence. Total: \$750K-\$1.2M.

Multi-Threaded Pursuit:

- Thread 1: SVP Strategic Transformation - organizational redesign. Angle: "How is the post-take-private operating model different from what worked as a public company?"
- Thread 2: SVP Supply Chain Services - value creation execution. Angle: "How are you coordinating the committed buying program rollout across 4,400 members?"
- Thread 3: Patient Square Capital operating partners - PE sponsor introduction. If McChrystal has PE network connections, a sponsor-level introduction creates top-down mandate. Success at Premier could replicate across Patient Square's ~\$13B portfolio.